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GUIDELINE FOR SUBMISSION OF EXTENSION APPLICATIONS

This guideline is intended to provide guidance to applicants who intend to submit extension applications for registration. It represents the South African Health Products Regulatory Authority's (SAHPRA) current position on the safety, quality and efficacy of these medicines.

It is not intended as an exclusive approach; alternative approaches may be used but these should be scientifically and technically justified. SAHPRA reserves the right to request any additional information to establish the safety, efficacy, and quality of a medicine in keeping with the knowledge current at the time of evaluation. SAHPRA is committed to ensure that all registered medicines will be of the required quality, safety and efficacy. It is important that applicants adhere to the administrative requirements to avoid delays in the processing and evaluation of applications.

Document History

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2		

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Glossary

Abbreviation/ Term	Meaning
API	Active Pharmaceutical Ingredient
BA	Bioavailability
BE	Bioequivalence
EA	Extension Application
EMA	European Medicines Agency
EU	European Union
FDC	Fixed Dose Combination
MAH: Equivalent to HCR	Market Authorisation Holder (equivalent to Holder of the Certificate of Registration)
MHLW	Ministry of Health, Labour and Welfare (Japan)
MHRA	Medicines and Healthcare Products Regulatory Agency (United Kingdom)
NCE	New Chemical Entity
PI	Professional Information
PIL	Patient Information Leaflet: Equivalent to Package Leaflet
PK	Pharmacokinetics
Q & BE	Quality and Bioequivalence
RRA	Recognised Regulatory Authorities – a term used to refer to the list of regulatory authorities with which SAHPRA aligns itself
SAHPRA	South African Health Products Regulatory Authority
SmPC	Summary of Product characteristics: Equivalent to PI
TGA	Therapeutic Goods Administration (Australia)
The Authority	Relevant regulatory authority, in this case SAHPRA
US FDA	United States Food and Drug Administration

1. INTRODUCTION

This guideline outlines the information required for the registration of extension applications. The information submitted will be evaluated in terms of the provisions of the Medicines and Related Substances Act, 1965 (Act 101 of 1965) (hereafter “the Act”).

Extension applications refer to applications for the registration of a medicine that is related to an already registered medicine. These applications may be submitted by the same Holder of the Certificate of Registration (HCR) and are referred to as line extension applications.

A line extension application is an application where for instance the pharmaceutical form and/or strength differs from one or more other medicines for which the applicant already holds a registration certification. For example, if an HCR holds a registration for paracetamol 500 mg tablets and applies for paracetamol 250 mg syrup, this will be classified as a line extension.

Applications submitted by a different HCR will be considered as extension applications, provided they fall within the categories outlined below.

Extension applications fall within these two main categories:

1. Changes to the active substance(s)
2. Changes to strength, pharmaceutical form and route of administration

These changes may be associated with changes related to therapeutic indication(s) and/or posology, assuming that the proposed population is included in the indication. These extension applications will be regarded as generics with clinical data. Applicants should refer to the Clinical Guideline for more detail.

It is a legal requirement that data submitted for evaluation should substantiate all claims and should meet technical requirements of quality, safety and efficacy of the product for the purposes for which it is intended. The guideline is meant to guide the applicant in meeting the requirements of the Act. It is acknowledged, however, that in some instances scientific developments may dictate alternative approaches. When a deviation from a guideline is decided on, a detailed motivation giving the reason(s) for the deviation and justification for the alternative approach should be included in the expert report submitted with the application.

1.1 Purpose

The aim of this document is to provide guidance to applicants on the appropriate data for extension applications for the registration of medicines for human use.

1.2 Scope

The guideline is applicable to categories A: Human Medicines, as defined in the Medicines and Related Substances Act (Act 101 of 1965). In addition to this guideline, SAHPRA reserves the right to request any

additional information to establish the safety, quality, and efficacy of a medicine in keeping with the knowledge current at the time of evaluation.

Requirements for Therapeutic equivalence

(1) A medicine is considered therapeutically equivalent to another medicine if both medicines -

(a) are -

(i) pharmaceutically equivalent, in that they contain the same amount of active substances in the same dosage form, meet the same or comparable standards and are intended to be administered by the same

route; or

(ii) pharmaceutical alternatives, in that they contain the same Active Pharmaceutical Ingredient (API) but differ either in chemical form of that API or in the dosage form or strength; and

(b) after administration in the same molar dose, their effects with respect to both efficacy and safety are essentially the same.

(2) Therapeutic equivalence is determined from comparative bioavailability, pharmacodynamic, clinical or in vitro studies which meet the requirements and accepted criteria for bioequivalence as determined by the Authority.

2. LEGAL PROVISION

The registration of a medicine in South Africa is governed by the provisions and requirements of the Medicines and Related Substances Act, 1965 (Act No. 101 of 1965), and the regulations and guidelines published in terms thereof.

3. EXTENSIONS

Extension applications refer to medicines based on a registered API. These medicines typically contain the same active substances as the registered reference medicine and generally fall within the same therapeutic class, and they may serve as therapeutic alternatives.

3.1 Types of Extensions

Categorization and definitions of the common types of extensions and how they relate to the registered medicine.

3.1.1 Changes to the active substances

The following changes to the active substance may be regarded as extension applications:

- I. Replacement of the active substance(s) by a different salt/ester complex/derivative (with the same therapeutic API) where the efficacy/safety characteristics are not significantly different. Example: replacing hydrochloride salt with a citrate salt of the same active substance.
- II. Replacement by a different isomer, a different mixture of isomers, of a mixture by an isolated isomer

where the efficacy/safety characteristics are not significantly different. Example: substituting a racemic mixture with a single enantiomer.

Such changes must retain the same active substance(s) and demonstrate that the altered compound does not significantly impact the efficacy or safety profile of the product.

3.1.2 Changes to strength, pharmaceutical form and route of administration

The following applies to extension applications related to strength, dosage form, and route of administration.

- i. Change of bioavailability
- ii. Change of pharmacokinetics (e.g., change in rate of release)
- iii. Change or addition of a new strength/potency
- iv. Change or addition of a new pharmaceutical form
- v. Change or addition of a new route of administration

3.1.2.1 Changes to bioavailability and pharmacokinetics:

Changes to the strength, pharmaceutical form or route of administration may significantly impact a medicine's bioavailability and PK characteristics. For instance, altering the release rates (e.g., from immediate release to modified release). These changes must be evaluated through a risk-based approach, with appropriate clinical and/or bioequivalence data submitted.

3.1.2.2 Changes to strength

Changes to strength refer to modifications in the amount of active substance per unit dose or concentration. The quantitative composition in terms of active substance represents the strength. The concept of strength and the concept of concentration are inherently linked. Strength represents the amount of active substance in the pharmaceutical form, which can be defined per unit dose or as a concentration. The concentration can be stated per unit of mass (e.g. 250 mg/g) or per unit of volume (e.g. 2 mg/mL) or in percentage (e.g. 5%) for medicines for which the strength has traditionally been expressed in such way. An extension application may have the same active substance but different strength from the registered reference product.

3.1.2.3 Changes to the pharmaceutical form and route of administration:

A pharmaceutical form is used to describe the manufactured product (i.e. the item as presented in the packaging by the manufacturer) or the pharmaceutical product (i.e. the item as intended to be administered to the patient, after any necessary transformation has been carried out). Changes to the pharmaceutical form thus, refers to modifications of the physical form of the medicine such as tablets, capsules, suspensions, or other dosage forms. For example, a registered 500 mg tablet vs. a new application for a 500 mg oral suspension.

The route of administration refers to the path by which the medicine taken into or applied to the body to achieve its intended effect. A change to the pharmaceutical form and/or route of administration may have a significant impact on the pharmacokinetic (PK) and pharmacodynamic (PD) profile of the medicine. Example: changing from oral to parenteral.

An extension application may thus have the same active substance as the registered medicine but differ in dosage form or formulation. Such extensions may differ from the registered medicine in administration and/or dosage.

4. Combination Medicines

For new combination medicines, it is necessary to take three factors into account:

- The safety and efficacy of the individual active substances.
- The safety and efficacy of the simultaneous use of the individual active substances (multi-component regimen); and the medical and scientific rationale should support the simultaneous use of more than one active substance.
- The possible chemical and physical interactions between active substances derived from a multi-component regimen when they are formulated into an FDC product.
- The APIs have suitable PK and Physiochemical properties.

These applications can be considered as extension applications, but the type of application will be a generic with clinical data or new chemical entity (NCE) depending on whether all APIs are registered or not, changes to the active substance(s) and changes to the strength, pharmaceutical form and route of administration.

N.B. Applications for extension in which the API of the original/innovator is not registered with SAHPRA will be regarded as NCEs and innovators, not an extension.

4.1 Additional data which may be required in support of extensions applications

In general, the clinical data from the original/innovator product may be provided by the applicant in support of the extension application. However, the data may not be sufficient to justify the extension applications, resulting in additional data being required to support the submission.

The following includes, but is not limited to, the data that may be required to support extension applications.

Types of extensions	Additional data usually required
Different salt/ester complex/derivative (with the same therapeutic API)	Evidence that there is no change in the pharmacokinetics of the API, pharmacodynamics and/or in toxicity which could significantly change the safety/efficacy profile (otherwise, to be considered as a new active substance)
Different route/pharmaceutical form (For parenteral administration, it is necessary to distinguish between intra-arterial, intravenous, intramuscular, subcutaneous and other routes)	Clinical data (safety/efficacy), pharmacokinetics, pre-clinical (e.g. toxicology), if justified
New route of administration	
New pharmaceutical form (same route)	

(conventional to modified)	
Different strength, same route/ pharmaceutical form and posology	Bioavailability studies
Suprabioavailable medicines Same dosage intervals but reduced doses intended to achieve same plasma/blood concentrations as a function of time.	Bioavailability studies
Active substances associated in a different proportion/different posology or if one or more is intended for modified release.	Clinical studies comparing existing/new proportion or dosage regimen, including bioavailability studies.

5. REFERENCES

The following related documents are referenced:

Department of Health (South Africa), 2021. *Policy for classifying medicines into therapeutic classes for purposes of therapeutic interchange*. Version 9.0. 6 July 2021. Pretoria: Department of Health.

European Commission, Directorate-General for Health and Food Safety, 2019. *Notice to applicants, Volume 2C: Regulatory guidelines – Guideline on the categorisation of extension applications (EA) versus variations applications (V)*. Revision 4. July 2019. Brussels: European Commission.

European Medicines Agency, 2012. *Reflection paper on considerations given to designation of a single stereo isomeric form (enantiomer), a complex, a derivative, or a different salt or ester as new active substance in relation to the relevant reference active substance*. EMA/651649/2010. 18 October 2012. London: European Medicines Agency.

6. VALIDITY

This guideline is valid for a period of 5 years from the effective date. It will be reviewed on this timeframe or as and when required.